Reviewer 1 v.1

Comments to the Author

This is a sistematic review and metanalysis comparing the diagnostic yields of virtual vs non virtual assisted bronchoscopic samplings in peripheral lung nodules. The paper is well written, methodologically appropriated and appears to include all the relevant RCT available in the literature.

Here are my suggestions and raised issues:

1) Authors evaluated the diagnostic yield according to many clinical and operational outcomes; this approach is quite similar to those used by previous works. I believe that, considering the recent onset of molecular analysis for personalized approach to lung cancer, they should have included also the appropriateness of samplings in providing enough tissue for molecular tumor assessment. Similarly, outcoming the goodness of the sample for semisolid, solid or GG lesion's profile would have been appropriate.

2) Authors found that VBNA provides a statistically significant shortening of the total examination time. I believe it is important to remark that this finding is far from being clinically relevant, I suggest authors to clarify this in the discussion section.

3) Considering that it is relevant, I would move the figure S1 from supplementals to main manuscript and would comment it more in deep.

4) Considering the conclusions of this metanalysis, I believe that authors should discuss the many issues in favour and/or against the use of virtual supported approaches vs other non virtual assisted ones, i.e. economic burden, availability, etc.

5) Still in the discussion, I would raise the concern related to the "expertise bias" that characterizes many studies evaluating diagnostic yield in bronchial endoscopy and whose diagnostic accuracy probably reflects the skillfulness of the interventional pulmunologists carrying out the study. Their extremely high diagnostic yield is unlikely to be generalizable to the real world; this is particularly true for fluoroscopy or radial EBUS, guide sheat guided sampling. This potential overestimation might affect the results of the metanalysis.

6) not considering some studies because "outlier" is an uncorrect approach that may provide some misleading message. If the authors decided to include this paper in their analysis, no exceptions should be made when interpreting their results. I would change this section according.

Minor issues:

1) use the term Systematic instead of Systemic in both the title and the text